
Patient: RIVERA, MARCUS **DOB:** 03/14/1987 **MRN:** LBH-40217 **Unit:** Adult Inpatient Psychiatry

INITIAL PSYCHIATRIC ASSESSMENT — 06/14/2026

REGISTRATION & IDENTIFYING INFORMATION

Marcus Rivera is a 39-year-old married male admitted to the Adult Inpatient Psychiatry unit on 06/14/2026 at 9:20 AM. Insurance: Blue Cross Blue Shield of Illinois PPO. Emergency contact: Elena Rivera (spouse).

Legal Status: Voluntary admission; consent to treatment signed by the patient at registration.

ADMISSION DIAGNOSES

Psychiatric: Alcohol use disorder, severe (F10.20); Major depressive disorder, moderate, single episode (F32.1). Concurrent medical conditions: essential hypertension (I10), untreated; elevated transaminases (R74.01) with AST 68 and ALT 54 on outpatient labs obtained three months prior.

PRESENTING PROBLEM

The patient was admitted for stabilization of escalating daily alcohol consumption accompanied by worsening depressive symptoms and passive suicidal ideation, occurring in the setting of marital and occupational stressors. Inpatient admission was judged clinically appropriate for supervised withdrawal management and initiation of structured treatment.

COURSE OF ILLNESS

Symptom onset traces to approximately three years ago, when drinking escalated from social use to 8-12 standard drinks daily following a stressful job change. Mood has declined progressively over the past six months with anhedonia, poor sleep, and hopelessness. Two weeks before admission his wife stated she would separate if he did not seek treatment; one week before admission his employer issued a formal warning and placed him on a 30-day performance improvement plan. Three days before admission, after a weekend binge, he experienced passive suicidal ideation without plan or intent and presented voluntarily at the urging of his wife and outpatient counselor.

MENTAL STATUS FINDINGS

Adequately groomed male appearing stated age with mild bilateral hand tremor. Cooperative, fair eye contact, normal speech. Mood "low" with constricted, congruent affect. Thought process linear and goal-directed. Passive suicidal ideation without plan or intent; no homicidal ideation, delusions, or hallucinations. Alert and oriented x4; attention and memory grossly intact. Insight fair, judgment fair. Estimated intellectual functioning average to above average.

PSYCHOSOCIAL REVIEW

Raised in Chicago, second of three children; father had alcohol use disorder. Bachelor's degree in business; employed six years as a project manager at a logistics firm. Married 11 years with two children ages 6 and 9. Discharge housing plan is return to the family home, which is stable with no safety concerns. Spouse was interviewed 06/15/2026: she supports treatment and will attend family sessions, though she reports exhaustion and states the marriage depends on his sobriety. Community resources: contact information provided for the Lakeview Alcoholics Anonymous group and a referral initiated to the Northside Intensive Outpatient Program; the outpatient counselor was contacted with patient consent.

SUBSTANCE HISTORY

Alcohol: 8-12 standard drinks daily for approximately three years; first use at age 16. One prior withdrawal episode in 2023 (tremor, sweating, anxiety), self-resolved. Morning tremor currently present. Completed a 12-week outpatient program in 2023 with relapse within four months. Cannabis: occasional social use. Tobacco and other substances: denied. DAST-10 score 6; AUDIT-C positive.

SPIRITUAL AND CULTURAL CONSIDERATIONS

Not addressed at intake.

EVALUATION TIMING

This psychiatric evaluation was completed 06/15/2026 at 2:30 PM by the attending psychiatrist, within 60 hours of admission on 06/14/2026 at 9:20 AM.

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INDIVIDUALIZED PLAN OF CARE — 06/15/2026

CONFIRMED DIAGNOSES

(1) Alcohol use disorder, severe (F10.20), supported by three years of daily heavy use, a prior withdrawal episode, examination tremor, elevated transaminases (AST 68, ALT 54), a failed outpatient treatment with early relapse, and continued use despite marital and occupational consequences. (2) Major depressive disorder, moderate (F32.1), supported by six months of depressed mood, anhedonia, insomnia, hopelessness, and passive suicidal ideation on mental status examination.

IDENTIFIED PROBLEMS

Problem 1: Physiological alcohol dependence with morning tremor and prior withdrawal. Problem 2: Depressed mood with passive suicidal ideation. Problem 3: Impaired occupational functioning under a 30-day performance improvement plan. Problem 4: Marital discord secondary to substance use. Problem 5: Untreated hypertension and alcohol-related transaminase elevation. Problem 6: History of relapse within four months of completing prior treatment.

GOALS OF CARE

During hospitalization: complete alcohol withdrawal management without complication with CIWA-Ar below 8 for 48 consecutive hours; reduce PHQ-9 from 16 to below 10 by discharge; remain free of suicidal ideation for 72 hours before discharge with a completed written safety plan. Following discharge: sustained abstinence at 6 and 12 months supported by naltrexone adherence and intensive outpatient participation; return to full occupational functioning; re-engagement in the marital relationship through at least six couples-focused aftercare sessions.

PLANNED INTERVENTIONS

Supervised alcohol withdrawal per CIWA-Ar protocol with symptom-triggered lorazepam. Naltrexone 50 mg by mouth daily beginning 06/16/2026 after liver function review. Cognitive behavioral therapy group 60 minutes daily. Motivational interviewing individually twice weekly. Relapse-prevention psychoeducation group three times weekly. One structured family session with the spouse before discharge. Lisinopril 10 mg daily for hypertension with a repeat liver panel.

CARE TEAM SIGNATURES

R. Okafor, MD — Attending Psychiatrist	06/15/2026
J. Lindqvist, RN — Charge Nurse	06/15/2026
D. Marsh, LCSW — Social Worker	06/15/2026

A. Chen, CADC — Addictions Counselor

06/15/2026

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NURSING PROGRESS NOTE — 06/14/2026

OBSERVATIONS

Patient states he is "nervous but glad to be here." Last drink reported approximately 14 hours before admission. Vital signs stable; mild bilateral hand tremor; CIWA-Ar score 6. Oriented x4 and cooperative on the unit.

CLINICAL IMPRESSIONS

Early mild alcohol withdrawal. Mood depressed but engaged with the admission process; no suicidal ideation expressed this shift.

PLAN OF CARE

Continue CIWA-Ar monitoring every 4 hours, encourage fluids, orient to unit schedule; psychiatric evaluation scheduled with Dr. Okafor.

Patient: RIVERA, MARCUS **DOB:** 03/14/1987 **MRN:** LBH-40217 **Unit:** Adult Inpatient Psychiatry**CLINICAL PROGRESS NOTE — 06/21/2026****INTERVAL DATA**

Withdrawal management completed without complication; CIWA-Ar below 8 since 06/18/2026 and the protocol was discontinued. Attended all CBT groups this week and both individual motivational interviewing sessions. Naltrexone tolerated without side effects. PHQ-9 today 12, down from 16 at admission. Wife visited 06/20/2026; the interaction was observed to be warm.

CLINICAL IMPRESSIONS

Meaningful early progress against short-term goals: the withdrawal goal is met, depressive symptoms are improving, and engagement is strong. Ambivalence about long-term abstinence persists; the patient stated in group that he believes he "could probably drink normally someday."

PLAN OF CARE

Continue current interventions. The team recommends adding a relapse-prevention focus on workplace triggers in individual sessions. No revision to plan goals at this time; PHQ-9 to be repeated in one week.

Patient: RIVERA, MARCUS **DOB:** 03/14/1987 **MRN:** LBH-40217 **Unit:** Adult Inpatient Psychiatry**CLINICAL PROGRESS NOTE — 07/14/2026****INTERVAL DATA**

Patient remains abstinent on the unit; naltrexone continued at 50 mg daily. PHQ-9 today 8. Attended a family session with his spouse on 07/11/2026, described by both as productive. Patient states: "I told Elena everything. She didn't blow up. I think that means something."

CLINICAL IMPRESSIONS

Depressive symptoms are now below the short-term goal threshold; a safety plan has been drafted and reviewed. The patient verbalizes commitment to intensive outpatient step-down and AA attendance. Progress is consistent with the plan of care; discharge planning is appropriate to begin.

PLAN OF CARE

Recommend revising the plan of care to reflect the discharge preparation phase: finalize the intensive outpatient intake date, complete the written safety plan, and schedule discharge medication reconciliation.

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DISCHARGE SUMMARY — 07/18/2026

HOSPITAL COURSE

Mr. Rivera was admitted voluntarily on 06/14/2026 for severe alcohol use disorder with early withdrawal and moderate major depressive disorder with passive suicidal ideation. He completed symptom-triggered withdrawal management by 06/18/2026 without complication. Naltrexone 50 mg daily was initiated and tolerated. He participated consistently in daily CBT group, twice-weekly motivational interviewing, and relapse-prevention psychoeducation, and completed one family session with his spouse. Depressive symptoms improved steadily (PHQ-9 16 at admission, 8 before discharge) and suicidal ideation resolved by the second week. Hypertension was managed with lisinopril 10 mg daily; a repeat liver panel on 07/10/2026 showed improving transaminases (AST 51, ALT 44).

STATUS AT DISCHARGE

At discharge on 07/18/2026 the patient is medically stable, abstinent from alcohol for the duration of the admission, and denies suicidal ideation, with a completed written safety plan. Mood is euthymic with brighter affect; insight and judgment are improved. He is motivated for continued treatment and has family support at home.

AFTERCARE INSTRUCTIONS

Intake at the Northside Intensive Outpatient Program on 07/21/2026, three evenings weekly. Continue naltrexone 50 mg daily with psychiatry follow-up in two weeks. Continue lisinopril 10 mg daily with primary care follow-up within 30 days including a repeat liver panel. Attend Alcoholics Anonymous at least twice weekly. Couples-focused family sessions through the intensive outpatient program. Return to the emergency department if suicidal ideation recurs; the crisis line number is on the written safety plan.